

When state Medicaid programs covered Wegovy and Zepbound, prescriptions rose by an estimated 12.2 per 1,000 enrollees per quarter

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12.2

estimated additional prescriptions per 1,000 enrollees per quarter (95% CI 8.4 to 16.0)

128.8M

U.S. adults meet the FDA label criteria (lower bound); 17.8M have Medicaid

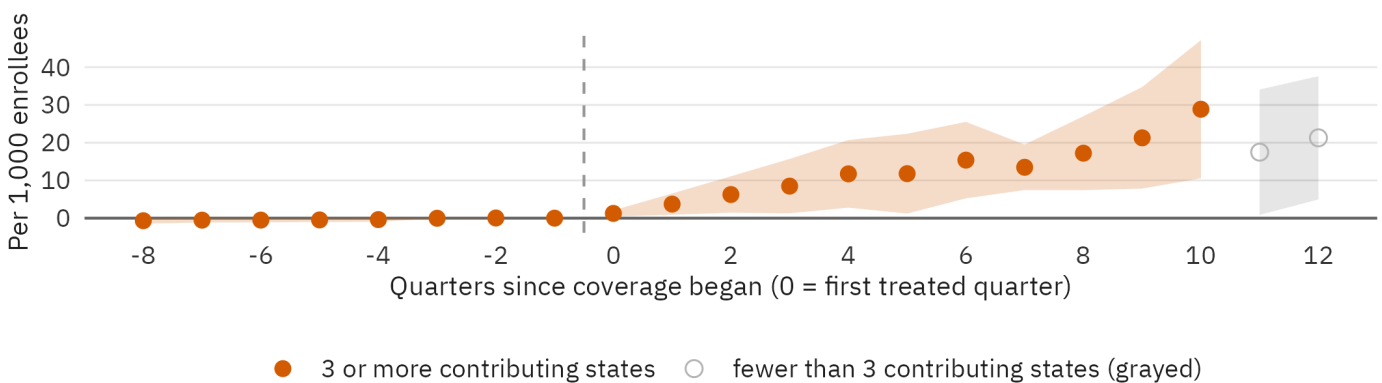
\$161.3M

five-year net cost, 1 million enrollees (central case); \$2.69 per member per month

All 15

estimable alternative analyses give the same answer (interval excludes zero)

Estimated effect of coverage on prescriptions, by quarter since coverage began



Source: CMS State Drug Utilization Data and Medicaid enrollment; 10 covering and 34 never-covering jurisdictions. Part of the growth is market growth.

What I did

- Public data**
CMS Medicaid and Medicare Part D data, NHANES, MEPS, KFF polls, FDA labels, state Medicaid documents.
- PostgreSQL and dbt**
54 models and 126 tests; NDC and ICD-10 coding; every number traced to a key-numbers file.
- R analysis**
Staggered difference-in-differences, survey-weighted eligibility, budget impact model with probabilistic sensitivity analysis.
- Quarto and Shiny**
Report, deck and an interactive budget model that reproduces the report's numbers.

Skills shown

- Real-world evidence
- HEOR
- Market access
- Causal inference
- Budget impact analysis
- Claims data and NDC coding
- SQL and dbt
- R, Quarto and Shiny

Budget impact: \$161.3 million net over five years (scenario range \$55.6 to \$369.4 million; rebate assumed). Rebate and years 3 to 5 are assumptions; scenarios, not forecasts.

Site: erickyegon.github.io/incretin-access-value/

Report: erickyegon.github.io/incretin-access-value/report.html

Deck: erickyegon.github.io/incretin-access-value/deck.pdf

Interactive budget model: 01a108a8-

ecde-397c-5353-39196812b10c.share.connect.posit.cloud/

Code: github.com/erickyegon/incretin-access-value

Research design pack: erickyegon.github.io/incretin-access-value/research_pack.pdf

Public aggregate data; no company affiliation or endorsement; not patient-level claims; gross of rebates unless stated; scenarios, not forecasts. I used AI tools to help write code and documentation. The study design, methods and conclusions are my own, and I verified all results.